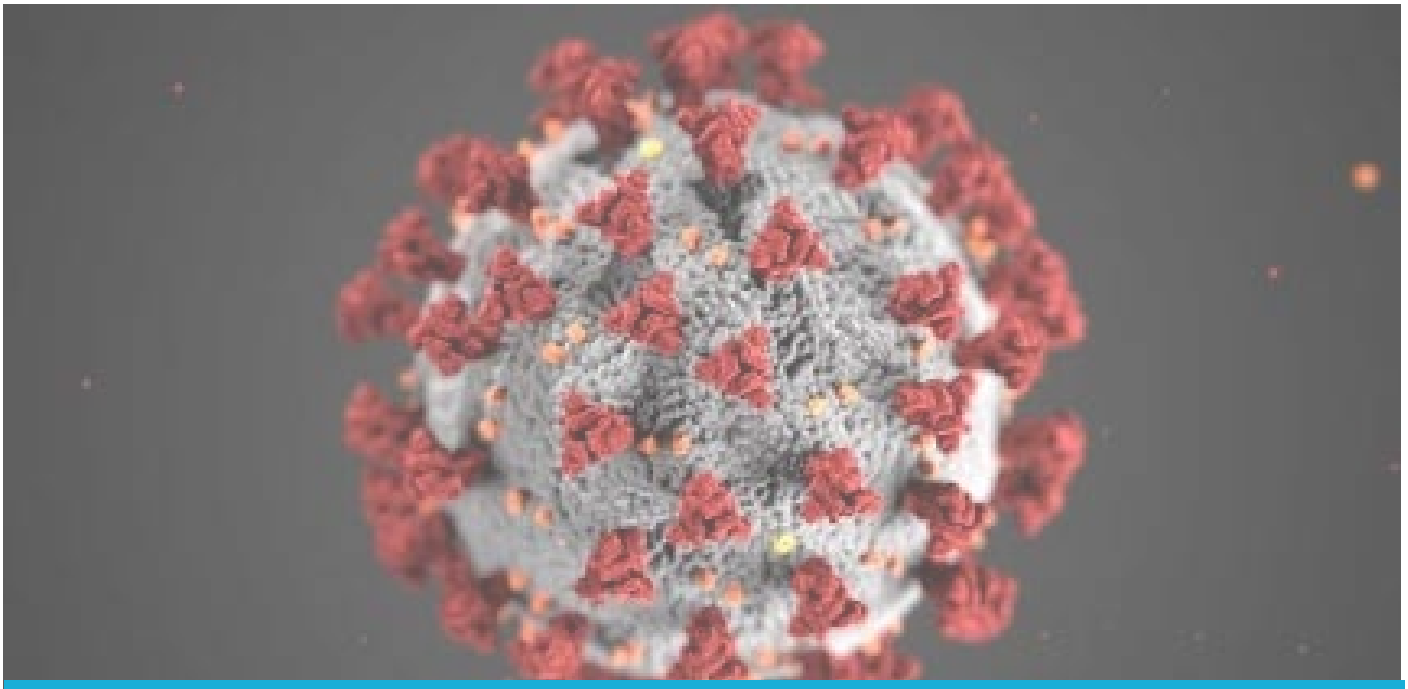


COVID-19 Vaccination Program Interim Operational Guidance Jurisdiction Operations

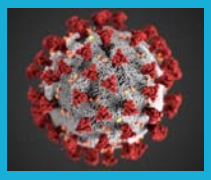
Centers for Disease Control and Prevention (CDC)

October 29, 2020

Version 2.0

U.S. Department of Health and Human Services
Centers for Disease Control and Prevention

Exhibit B



Section 1: Public Health Preparedness Planning

Pandemic vaccination response planning requires collaboration among a wide range of public- and private-sector partners, including immunization and public health emergency preparedness programs, emergency management agencies, healthcare organizations, industry groups that include critical infrastructure sectors, policy makers, and community vaccination providers (e.g., pharmacies, occupational health settings, doctors' offices). Many of these partners are engaged regularly in seasonal influenza and other outbreak vaccination campaigns, and many served as vaccination providers[1] during the 2009 H1N1 pandemic. However, significant additional planning is needed to operationalize a vaccination response to COVID-19, which is much larger in scope and complexity than seasonal influenza or other previous outbreak-related vaccination responses. Following the planning and improvement guidance below can assist in developing a jurisdiction's baseline readiness to launch the COVID-19 Vaccination Program.

Improvement Planning

Improvement planning is the identification of strengths, areas for improvement, and corrective actions that results from workshops, exercises, or real-world events. Jurisdictions should use a consistent approach for improvement-related activities across each of their COVID-19 vaccination preparedness planning components. Gaps in program planning are often identified when plans are tested, whether through a real event, such as a hepatitis A outbreak, or a full-scale vaccination exercise. Jurisdictions should test their COVID-19 vaccination program plans, and after testing, assign roles and responsibilities with target completion dates for specific tasks to ensure that corrective actions are fully implemented. Periodic review and revision of plans are integral to the improvement process. Jurisdictions should support continuous quality improvement as they move through different phases of workshops, exercises, and actual COVID-19 vaccination program implementation, making and operationalizing improvements in an ongoing manner.

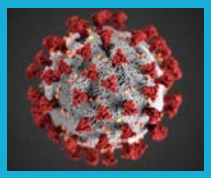
COVID-19 Vaccination Program Planning

Prior to plan development, it is important for jurisdictions to have full situational awareness. There are many unknowns and unanswered questions at this time. For example, it is not yet known which vaccines will be available, in what volumes, at what time, with what efficacy, and with what storage and handling requirements. However, jurisdictions should review all current planning assumptions to assist with early planning efforts. (See *Appendix A: COVID-19 Vaccination Planning Assumptions for Jurisdictions.*)

In addition to current situational awareness, there is much to learn from past experiences. Jurisdictions may find it helpful to review their 2009 H1N1 pandemic vaccination response plans and lessons learned. After-action reports and improvement plans from that time provide an opportunity for jurisdictions to build on prior strengths and determine any gaps that may need to be addressed.

After plans have been drafted, it is important to identify any weaknesses by conducting exercises, including workshops or tabletop, functional, or full-scale exercises. This may be particularly valuable for any activities planned with external partners. For example, vaccination clinics in temporary or off-site settings, such as those planned for essential workers, are an excellent opportunity for exercises. Specific procedures to assess could

[1] For the purposes of this document, "vaccination provider" refers to any facility, organization, or healthcare provider licensed to possess/administer vaccine or provide vaccination services. A "COVID-19 vaccination provider" is any vaccination provider who has been enrolled in the COVID-19 Vaccination Program.



Section 2: COVID-19 Organizational Structure and Partner Involvement

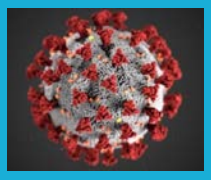
Pandemic vaccination planning is a combined state, territorial, tribal, and local responsibility that requires close collaboration between public health, external agencies, and community partners. Depending on the specific jurisdiction's governance structure, the jurisdiction may play a key role in ensuring readiness at all levels. It is imperative that state and local jurisdictions, tribal nations, and their planning partners clearly understand each other's roles and responsibilities in the COVID-19 Vaccination Program.

Planning and Coordination Team (Internal)

An internal COVID-19 Vaccination Program planning and coordination team is critical to ensure the vaccination response to COVID-19 is thoughtfully planned and successfully executed. A wide array of expertise should be represented among team members. Jurisdictions should consider broad inclusion from the immunization program, preparedness program, legal affairs, media/public affairs, and crisis and emergency risk communication (see *Section 12: COVID-19 Vaccination Program Communication* for additional information on crisis communication planning). In addition, the team should include clinical expertise as well as representatives of programs that serve the early populations of focus (e.g., Bureau of Aging, state licensing board, HIV/AIDS program, rural health office). It may even be helpful to include representation from a local public health jurisdiction for implementation perspective. Team members should be assigned responsibilities based on their individual expertise to best enhance plan development and activities coordination before and during the response. To mitigate any unexpected situations affecting a team member, each team member should have a backup representative to ensure coverage of each specialty area remains intact throughout the COVID-19 Vaccination Program. For any roles that are not adequately staffed, efforts should be made to recruit for and fill any team member vacancies as early as possible.

State-Local Coordination

State governance structures vary from centralized to decentralized. In a centralized state, legal authority is concentrated in the central state government, which makes decisions and performs most functions. Conversely, in a decentralized state, authority and responsibilities are dispersed and distributed across regions and areas. Regardless of the jurisdiction's governance structure, it is imperative that state and local authorities combine and coordinate efforts. State-level personnel must closely monitor activities at the local level to ensure the COVID-19 Vaccination Program is implemented throughout the jurisdiction in adherence with federal guidance and requirements, and that there is equitable access to COVID-19 vaccination across all areas. Local personnel likely have a better understanding of perceptions, unique challenges, and successful mitigation strategies within their communities. Aligning areas of responsibility as well as specific tasks can help to complement rather than duplicate efforts at either level, maximizing the efficient use of resources and overall quality of the COVID-19 Vaccination Program.



Tribal Nations and Tribal Communities

While engaging with tribal leaders, jurisdictional[2] immunization programs must remember each tribal nation has the sovereign authority to provide for the welfare of its people and, therefore, has the authority to:

- Choose among the jurisdiction or Indian Health Service (IHS) options for accessing vaccine.
- Determine the population(s) it chooses to serve.
- Choose how vaccines are distributed to its community.
- Establish priority groups when there is a limited supply of COVID-19 vaccine or other accompanying resources.

For the COVID-19 Vaccination Program, tribal nations have two options for receiving vaccine:

1. Through the jurisdiction's allocation and distribution mechanism
2. Through the IHS allocation and distribution mechanism

State and local jurisdictions do not possess legal authority over tribal nations directly providing vaccine to their service populations. However, if a tribal nation or any of the health facilities serving that tribal nation receive vaccine from the jurisdiction's allocation, they are responsible for adhering to vaccine storage, handling, distribution, and reporting requirements outlined in the *CDC COVID-19 Vaccination Program Provider Agreement.*

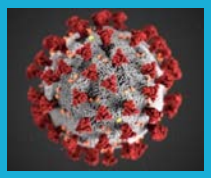
Jurisdictions should reach out to tribal nations within their respective areas for involvement in planning efforts. Jurisdictions must include each tribe's preference for COVID-19 vaccine distribution to ensure vaccine is effectively delivered to tribal nations and their communities. State and local jurisdictions should also engage with Urban Indian Health Centers (UIHCs). IHS may be able to support distribution to UIHCs and is planning to formally confer with UHICs to solicit their feedback. Additionally, awardee jurisdictions should reach out to UIHCs as part of the planning process to determine their preference for vaccine access. Details of engagement with tribal nations and other tribal entities should be included in jurisdiction COVID-19 vaccination plans.

The jurisdictional planning process should include state-recognized tribes, unrecognized tribes, and American Indian/Alaska Native individuals who are included in state-recognized tribes because the option to access COVID-19 vaccine through IHS may not be possible for these communities.

COVID-19 Vaccination Program Implementation Committee (Internal and External)

Reaching intended vaccine recipients is essential to achieving desired levels of COVID-19 vaccination coverage. To ensure equitable access to vaccinations, information about populations within a jurisdiction and the logistical requirements for providing them access to COVID-19 vaccination services will require collaboration with external entities and community partners who are familiar with how they obtain healthcare and other essential services. Jurisdictions should establish a COVID-19 Vaccination Program implementation committee to enhance development of plans, reach of activities, and risk/crisis response communication messaging and delivery. Committee membership should include leadership from the jurisdiction's COVID-19 planning and coordination team as well as representatives from key COVID-19 vaccination providers for critical population groups

[2] "Jurisdiction/jurisdictional," as used in this document, refers to the federal immunization funding awardees described in the Executive Summary and their state public health emergency preparedness counterparts who are tasked with developing COVID-19 vaccination plans for submission to CDC.

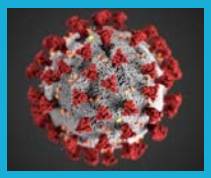


Vaccination Provider Enrollment

To receive/administer COVID-19 vaccine, constituent products, and ancillary supplies, vaccination provider facilities/organizations must enroll in the federal COVID-19 Vaccination Program coordinated through their jurisdiction's immunization program. Enrolled COVID-19 vaccination providers must be credentialed/licensed in the jurisdiction where vaccination takes place, and sign and agree to the conditions in the *CDC COVID-19 Vaccination Program Provider Agreement*. These conditions are detailed in the agreement itself:

1. Administer COVID-19 vaccine in accordance with ACIP recommendations. (*Note: ACIP will review data on the safety and efficacy of each available COVID-19 vaccine and vote on recommendations for use.)*
2. Within 24 hours of administering a dose of COVID-19 vaccine and adjuvant (if applicable), record in the vaccine recipient's record and report required information to the relevant state, local, or territorial public health authority. (See *Appendix D: CDC IIS Data Requirements for COVID-19 Vaccine Monitoring*). The provider must maintain the vaccine administration records for at least 3 years following vaccination, or longer if required by state, local, or territorial law. These records must be made available to any federal, state, local, or territorial public health department to the extent authorized by law.
3. Not sell or seek reimbursement for COVID-19 Vaccine and any diluent, syringes, needles, or other constituent products and ancillary supplies provided by the federal government.
4. Administer COVID-19 vaccine regardless of the vaccine recipient's ability to pay.
5. Provide an Emergency Use Authorization (EUA) fact sheet for recipients or vaccine information statement (VIS), as applicable, to each vaccine recipient/parent/legal representative prior to vaccination.
6. Comply with CDC requirements for vaccine management, including storage and handling, temperature monitoring at all times, complying with jurisdiction's instructions for dealing with temperature excursions, and monitoring expiration dates. Providers must keep all records related to COVID-19 vaccine management for a minimum of 3 years, or longer if required by law.
7. Report COVID-19 vaccines and diluents that were unused, spoiled, expired, or wasted as required by the jurisdiction's immunization program.
8. Comply with federal instruction regarding disposal of unused COVID-19 vaccine and diluent.
9. Report vaccine administration errors (whether associated with an adverse event [AE]) or not, serious AEs (irrespective of attribution to vaccination),[3] multisystem inflammatory syndrome (MIS) in children or adults, and cases of COVID-19 that result in hospitalization or death to the Vaccine Adverse Event Reporting System (VAERS). Report any additional AEs and adhere to any revised safety reporting requirements per the U.S. Food and Drug Administration's (FDA) conditions of authorized vaccine use posted on FDA's website throughout the duration of the EUA, as applicable. Healthcare providers should also report any additional clinically significant adverse events following COVID-19 vaccination to VAERS, even if they are not sure if the vaccination caused the event.
10. Provide a completed COVID-19 vaccination record card to every vaccine recipient/parent/legal representative.

[3] Serious AEs are defined as: death, a life-threatening AE, inpatient hospitalization or prolongation of existing hospitalization, persistent or significant incapacity or substantial disruption of the ability to conduct normal life functions, a congenital anomaly/birth defect, or an important medical event that may not result in death, be life-threatening, or require hospitalization when, based upon appropriate medical judgment, it may jeopardize the individual and may require medical or surgical intervention to prevent one of the outcomes listed above. Serious AEs should be reported regardless of causality.



11. Comply with FDA's requirements, including EUA-related requirements described in FDA's Letter of Authorization, as applicable. Providers must also administer COVID-19 vaccine in compliance with all applicable state and territorial vaccine laws.

Failure of any enrolled COVID-19 vaccination provider organization or vaccination location under its authority to meet the conditions of the agreement may impact whether COVID-19 vaccine product orders are fulfilled and may result in legal action by the federal government.

Enrolled COVID-19 vaccination providers must also fully complete the *CDC COVID-19 Vaccination Provider Profile* form for each location where COVID-19 vaccine will be administered. The profile form collects the following variables for each location:

- Address and contact information
- Days and hours of operation
- Vaccination provider type (e.g., medical practice, pharmacy, LTCF)
- Settings where vaccine will be administered (e.g., hospital, university, temporary or off-site clinic)
- Number of patients/clients served
- Influenza vaccination capacity during the peak week of the prior (2019–2020) influenza season
- Populations served (e.g., pediatric, adult, military, pregnant women)
- Current IIS reporting status
- Vaccine storage unit capacity in volume and ability to maintain required temperatures

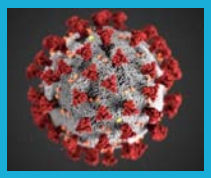
The profile form includes a field where the brand/model/type of storage unit is to be listed, requiring an attestation from the medical/pharmacy director or vaccine coordinator that each unit will maintain the relevant required temperatures (i.e., refrigerated [2°C to 8°C], frozen [-15° to -25°C], ultra-cold [-60° to -80°C]. If desired, the immunization program may request photos of vaccine storage units for confirmation.

Both forms (agreement and profile) may be submitted to the jurisdiction electronically, and it is permissible for an immunization program to develop and administer these forms in the IIS or other system.

*Note: A **vaccine coordinator** is the POC for receiving vaccine shipments, monitoring storage unit temperatures, managing vaccine inventory, etc. Immunization programs should encourage enrolled facilities/organizations to designate a vaccine coordinator role at each location as well as a back-up vaccine coordinator.*

Provider enrollment activities that immunization programs must complete include:

- Ensure provider agreement, profile form, and redistribution agreement (if applicable) are thoroughly and accurately completed by each enrolled provider, retained on file for a minimum of 3 years, and made available to CDC upon request
- Verify COVID-19 vaccination providers) have active, valid licensure/credentials to possess and administer vaccine. This licensure verification is needed only for those with prescribing authority [e.g., MD, DO, RPh, NP, PA] who will oversee COVID-19 vaccine administration. Credential verification is not required for vaccinators who work under the authority of someone with a higher level of licensure (i.e., not required for pharmacy techs/interns, RNs, LPNs, medical assistants, etc.).
- Onboard COVID-19 vaccination providers to the jurisdiction's IIS or other external system using an expedited process.
- Enter ship-to site information for each enrolled COVID-19 vaccination provider location in the Vaccine Tracking System (VTrckS) via direct upload or extensible XML information set (ExIS). *(Note: To support*



Federal Pharmacy Partnership for COVID-19 Vaccination in Long-Term Care Facilities

CDC will collaborate with CVS and Walgreens to provide on-site vaccination clinics for LTCF residents. CDC is working closely with LTCFs, jurisdictions, Centers for Medicare and Medicaid Services (CMS), professional trade organizations that serve nursing homes and assisted living facilities, and pharmacy partners to inform facilities of their options to receive COVID-19 vaccine. Depending on when LTCF staff is prioritized to receive vaccine, they will be covered under this plan (if prioritized at the same time as residents) or covered under jurisdiction plans for vaccinating healthcare workers/essential populations (if prioritized before residents). If staff is prioritized before residents, any staff not already vaccinated may be vaccinated through the on-site clinics offered by pharmacy partners. (See *Appendix H: Pharmacy Partnership for Long-Term Care Program for COVID-19 Vaccination.*)

Federal Direct Allocation to Pharmacy Partners (Phase 2)

To vaccinate a broader population group in Phase 2, vaccine will be allocated and distributed directly from the federal government to select pharmacy partners. Direct allocation opportunities will be provided to retail chain pharmacies and networks of independent and community pharmacies[4] (those with a minimum of 200 stores). All partners must sign a pharmacy provider agreement with the federal government. As part of such agreement, before receiving COVID-19 vaccine, the partner must propose, in writing, its minimum capacity for vaccine administration, including a) the number and location of facilities that will administer COVID-19 vaccine, b) the estimated number of COVID-19 vaccine doses that each facility will be able to administer within defined periods, and c) estimated cold chain storage capacity.

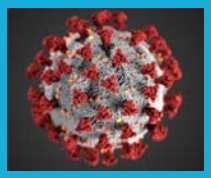
On a daily basis, pharmacy partners must report to CDC via designated methods the number of doses of COVID-19 vaccine a) ordered by store location; and b) on hand in each store reported through VaccineFinder. Pharmacy providers will also be required to report CDC-defined data elements related to vaccine administration to jurisdiction IISs. CDC will provide information on these data elements and reporting methods if stores are not able to directly provide data to jurisdiction IISs.

Partnerships with pharmacies will need to be synchronized with jurisdictions to improve vaccination coverage and ensure transparency across the COVID-19 Vaccination Program. Jurisdictions will be provided an opportunity to opt out of having pharmacies in their area receive direct allocations. Once a jurisdiction opts out, it will not be able to opt back into the program. All jurisdictions participating in this program will have visibility on vaccine supply and uptake data by store within their respective areas. (See *Appendix I: Federal Pharmacy Partnership Strategy for COVID-19 Vaccination Program*.)

Related Guidance and Reference Materials

HHS authorization for state-licensed pharmacists to administer vaccines

[4] Pharmacy services administrative organizations, or PSAOs



Section 9: COVID-19 Vaccine Administration Documentation and Reporting

CDC requires that vaccination providers enrolled in the COVID-19 Vaccination Program report certain data elements for each dose administered within 24 hours of administration. (See *Appendix D: CDC IIS Data Requirements for COVID-19 Vaccine Monitoring).* Jurisdictions should assess the capability of COVID-19 vaccination providers to meet federal and jurisdiction-specific reporting requirements before or upon enrollment. The required data elements are located on the ISD Awardees SharePoint site. COVID-19 vaccination providers may view the data requirements on CDC's IIS website. Jurisdictions should be prepared to provide additional support or technical assistance for smaller vaccination providers or rural clinic settings.

Jurisdictions must facilitate and monitor IIS reporting by enrolled vaccination providers. Each vaccination location should be ready (including trained staff, necessary equipment, and internet access) to report vaccine administration data to the IIS or other external system at the time of vaccination. If data will be entered off site, vaccination providers must ensure the required data are reported to the IIS or other designated system within 24 hours. Reporting data may be transmitted daily from the jurisdiction's designated system to the CDC via the IZ Gateway[7] "Connect" component. Additional information on the reporting process and specifications will be shared as soon as they have been finalized. Jurisdictions will not be responsible for reporting data from federal agencies or commercial partners who receive vaccine allocations directly from CDC.

In addition to reporting vaccine administration, jurisdictions must put processes in place to match first and second doses including addressing the need to exchange data with or query other jurisdiction's systems and/or the Immunization Data Lake[8] to obtain immunization history, if applicable.

Jurisdictions should ensure redundant measures and procedures are in place for recording vaccine administration data in instances of connectivity problems or failures in the jurisdiction's IIS or other system. The jurisdiction's IIS should collect, report, and submit data directly to CDC's Immunization Data Lake and jurisdictional reporting requirements. (Additional information on CDC data requirements is forthcoming.) Planning activities should include onboarding to IZ Gateway Connect and Share[9] (if feasible) components;

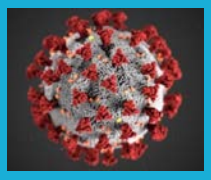
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[7] The Immunization Gateway (IZ Gateway) facilitates electronic messaging of vaccination records in a secure infrastructure allowing IIS systems across the nation to share vaccine administration data not only between jurisdictions, but also with provider organizations (e.g., Department of Defense, Federal Bureau of Prisons, IHS, Department of Veterans Affairs) that do not exchange data with the IIS today.

[8] The Immunization Data Lake is a cloud-hosted data repository to receive, store, and manage COVID-19 vaccination data for doses administered, vaccination coverage, ordering, inventory, and distribution. The Data Lake will provide a catalogue of different COVID-19 vaccine-related data sources that can be used to aid in monitoring COVID-19 vaccine ordering, distribution, coverage, and uptake. Data streams currently being onboarded to the Data Lake include provider enrollment data, VTrckS, and VaccineFinder.

[9] There are multiple ways to onboard to the IZ Gateway, including Connect and Share.

- **Connect** enables large national and non-traditional vaccination systems for satellite/temporary/off-site clinic settings to report and query immunization data with IISs, using the gateway's centralized data exchange, avoiding multiple individual, and point-to-point connections.
- **Share** allows exchange of immunization data between IIS jurisdictions by automating message triggers through the



Section 12: COVID-19 Vaccination Program Communication

Starting before COVID-19 vaccines are available, clear, effective communication will be essential to implementing a successful COVID-19 Vaccination Program. Building vaccine confidence broadly and among groups anticipated to receive early vaccination, as well as dispelling vaccine misinformation, are critical to ensure vaccine uptake.

A successful COVID-19 Vaccination Program will have lasting effects on the nation's immunization system and overall vaccination efforts in the future. Using risk communication principles along with the CDC's recently developed Vaccinate with Confidence framework, jurisdictions can develop and implement timely, evolving plans as the foundation for their overall COVID-19 vaccination communication efforts.

COVID-19 Vaccination Communication Objectives

- Educate the public about the development, authorization, distribution, and execution of COVID-19 vaccines and that situations are continually evolving.
- Ensure public confidence in the approval or authorization process, safety, and efficacy of COVID-19 vaccines.
- Help the public to understand key differences in FDA emergency use authorization and FDA approval (i.e., licensure).
- Engage in dialogue with internal and external partners to understand their key considerations and needs related to COVID-19 vaccine program implementation.
- Ensure active, timely, accessible, and effective public health and safety messaging along with outreach to key state/local partners and the public about COVID-19 vaccines.
- Provide guidance to local health departments, clinicians, and other hosts of COVID-19 vaccination provider locations.
- Track and monitor public receptiveness to COVID-19 vaccination messaging.

Key Audiences

Messaging should be tailored for each audience to ensure communication is effective.

- Healthcare personnel (i.e., organizations and clinicians who will receive information about receiving and administering vaccine)
- Health insurance issuers and plans (coverage for vaccine, in-network providers)
- Employers
- Government and community partners and stakeholders
- Public/consumers
    - Essential workers
    - Those in groups at risk for severe outcomes from COVID-19 infection
    - Those in groups at increased risk of acquiring or transmitting COVID-19
    - Those with limited access to vaccination services

Broad Communication Planning Phases

Messaging should be timely and applicable for the current phase of the COVID-19 Vaccination Program.

- Before vaccine is available
- Vaccine is available in limited supply for certain populations of early focus (Phase 1)